NCCP 00720 · VERSION 2

Neratinib · extended adjuvant treatment

Neratinib is taken once daily with food for one year. The usual starting dose is 240 mg; the team may interrupt or reduce it for toxicity. Treatment starts less than one year after completing adjuvant trastuzumab.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Diarrhoea, dehydration and kidney injury

☐ Nausea, vomiting and reduced appetite

☐ Liver injury

☐ Harm to an unborn baby

Discuss the exact setting, expected benefit, alternatives, medicine-specific risks, monitoring, contraception and urgent contact plan.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC monthly for the first six months, then every three months. Liver tests at week 1, monthly for the first three months, then every six weeks; test earlier for liver symptoms or significant diarrhoea. Confirm normal baseline LVEF eligibility; cardiac/LVEF assessment is clinically indicated. Review bowel symptoms, hydration, weight and interacting medicines. Continue any follow-up already required after prior trastuzumab; neratinib does not reset that plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.